جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عليه عبدالسلام سيد نصر
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** -
رقم البطاقة : 25806032100065 **رقم التليفون :** 01122811410 **تاريخ تسجيل الإستمارة :** 24-10-2026
العمر(تاريخ الميلاد) : 03-06-1958 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام رنين مغناطيسي علي المخ والحجاج بالصبغه [11112] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عليه عبدالسلام سيد نصر
National ID: 25806032100065

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced MRI of the brain and orbits

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	عايه عبد السلام سيد نصر 0	**Code Number**	YK 262930
Sex/Age	Female 67Y	**Patient ID**	20105
Registered Date	23/05/2026 14:24:38	**Referred By Dr.**	Surgery
Validation Date	20/06/2026 09:10:04	**Branch**	Gustave Roussy
Reporting Date	6/20/2026, 10:27:04 AM		

Pathology Report

Pathology Report

Clinical Data: Left breast cancer, post-NACT.

Nature of specimen: Left modified radical mastectomy.

Gross Examination:
* Left modified radical mastectomy specimen measured 28x18x5 cm, with covering skin ellipse measured 24x12 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site 5x2.5x2 cm, located 1 cm from the least deep margin.
* Axillary fat measured 8x7x2 cm, entangling 10 lymph nodes, the largest 1.5x1x0.5 cm.
* Block designation:
* 11 Blocks:
1:5- T.B.S., 6- N&A., 7- Deep margin., 8:11- ALNs.
- Gross was done by; Dr. Heba Hassan.

Microscopic examination:
Histological type: No residual viable remnants of the previously diagnosed invasive tumor. (Cellularity 0%).
Ductal carcinoma in situ: Absent.
Angioinvasion: Absent
Micro-calcification: Present.
Nipple and areola: Free.
Deep margin: Free.
Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10) (without therapy effect)
Histologic response to therapy: Pathological complete response to therapy.

Diagnosis:
Left Breast Cancer (Post-NACT); Left Modified Radical Mastectomy:
- No residual viable remnants of the previously diagnosed invasive tumor. (Cellularity 0%).
- Ductal carcinoma in situ: Absent.
- Nipple and areola: Free.
- Deep margin: Free.
- Axillary lymph nodes: Ten lymph nodes free of metastatic deposits (0/10) (without therapy effect).
- Histologic response to therapy: Pathological complete response to therapy.
Stage: ypT0, ypN0.

Signature
Dr. Walaa Ibrahim Ghoneim

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	عليه عبد السلام سيد نصر 0	**Code Number**	YK 262930
Sex/Age	Female 67Y	**Patient ID**	20105
Registered Date	23/05/2026 14:24:38	**Referred By Dr.**	Surgery
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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

04/29/2026

MDT FORM

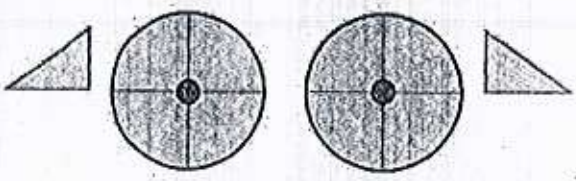
Name:	Alyia Abd Elsalam Sayed .	MRN:	687
Age:	68 Y	ID:	25806032100065
Menopausal Sta	POSTM	Phone:	01021508718
NRS SCORE:	PS2	Marital Status:	
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	

Clinical Data:

BX: IDC

ER: 6/8 PR: 3/8 HER2: 3+ KI67:

R L



TNM

Sonomammography:

Mets Workup:

History:

67 Y WT 80 HT160 SA 1.8 POSTM LEFT BREAST CANCER CT3 N2 MO ER6/8 PR3/8 HER23+ PT WAS DIAGNOSED IN 2024 BUT DELAYED BEC OF CAR ACCIDENT (RT FEMORAL FRACTURE AND INT FIXATION) CTS 11/2025 FREE BS 11/2025 ACTIVE UPTAKE AT STERNUM AND SC J FOR CT CORR CTS FREE RT FEMORAL TROCHANTER LESION (CAR ACCIDENT) PT RECEIVED 6 TAXOTERE CARBOPLATIN LAST 18/4/2026 3 PHESGO WITH REGRESSIVE COURSE FOR MDT FOR SURGERY

Recommendations:

MDT Outcome & Plan

for simple mastectomy and axilla according to response

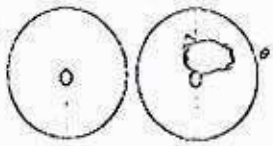
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/	د/	د/ نرمين رمضان			

MDT form

A-Patient ID:
1- Name : عليه عبدالسلام سيد نصر Sex: Female 3- Medical number: 83253140 Old Med : 43-2025

2-ID number: Patient Age : 67 Y



Family history	-ve	Phone	01122811410	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	03/02/2025
Co morbidity	htn	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
lt breast UOQ area of pleomorphic calcification with underlying focal syr
9.5x5 cm
multiple pathological LN

Mets Workup :
bulk uterus with thickened endometrium

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	6/8	3/8	3 +ve		35

T	N	M
CT3 M	1	0

Sonomammography

PATIENT HISTORY
Pt has Acc. accident Rt br stat NAC at this hosp
LABS, ECHO
D&C
BONE SCAN
CT CAP & bone sc → Fdg only uptake on breast at site of tum.. to start NACT TCHP X6

MDT OUTCOME & PLAN

NACT TCHPX6 THEN LT MASTECTOMY

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذة / عايدة عبد السلام سيد بدير		
Patient No.	: 25806032100065	Request No.	: 13002353
Path No.	: 4747/24	Request Date	: 03/12/2024
Sex	: Female	Verification Date	: 09/12/2024
Age	: 66 Y	Financial Category	: دور أول نائب السلام - مباشرة
Referreing Doc	:	Department	: Out Patient
		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data:
Left breast mass.

Nature of specimen:
Tru cut biopsy, collected at 2/12/2024 .

Gross examination
Three greyish white tissue cores up to 1.5 cm , totally submitted.

Microscopic examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II .
* DCIS:Focal.
*Angioinvasion: Could not be detected within the limits of biopsy recieved.

Diagnosis:
Left breast mass, tru cut biopsy:
Invasive duct carcinoma, Grade II .
For further IHC (ER,PR,HER2,Ki67).

Prof.Gamal Abdelatty
Dr.Noura Hefny

Verified by :
dr.esraa magdy

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / عليه عبد السلام سيد نصر	Request No.	: 13002353
Patient No.	: 25806032100065	Request Date	: 03/12/2024
Path No.	: 4747/24	Verification Date	: 25/12/2024
Sex	: Female	Financial Category	: اوراد دار السلام - مبادره
Age	: 66 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical data: Left breast mass.
Nature of specimen:
Tru cut biopsy, collected at 2/12/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 15% of tumor cells (score 6/8).
- PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 35% nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.
IPT:
- ER: Positive (score 6/8).
- PR: Positive (score 3/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 35% of tumor cells.

* Scoring system: "ER, PR"

Verified by :
Dr.Toka Medhat

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Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / عليه عبد السلام سيد نصر	Request No.	13002353
Patient No.	25806032100065	Request Date	03/12/2024
Path No.	4747/24	Verification Date	25/12/2024
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	66 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Salma Tarek
Dr. Duaa Mohamed
Dr. Yasmine Essam

Verified by :
Dr.Toka Medhat

CAP ACCREDITED
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Immunohistochemistry
[[DOCTYPE:IHC]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPTE

Name : ALIAA ABDEL SALAM SAYED
Branch : Portable
Ref.Physician Prof : TRIPHASIC
Date : 07-02-2026 12:19
Medical No. : GRI-94759
Ins.Company :

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER BOTH ORAL & I.V. CONTRAST ADMINISTRATION TRIPHASIC:

Findings:

In comparison to previoys study dated 8/11/2025 current study reveals:

- The uterus is mildly bulky with distended cavity, right adenxal cyst and dilated right fallopian tube with fluid inside ?Salpemgococle for US correlation.
- Small fatty umblical hernia.
- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Clear scanned lung bases.

Opinion:

- The uterus is mildly bulky with distended cavity, right adenxal cyst and ?Salpemgococle for US correlation.

Dr Mai Mohamed MD

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Alia Abdul Salam Sayyid Nasr
Date : 29-12-2025 10:01
Branch : Nasrcity
Medical No. : 248576
Ref.Physician Prof : Goustav Roussey Egypt
Ins.Company : التامين الصحي

..Whole body PET/CT EXAMINATION

Clinical History:

- 67 years old female patient diagnosed with left cancer breast received chemotherapy. PET/CT requested for initial assessment.

Technique:

- For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is < 2.5.



Report:

- **Left breast lower outer FDG avid metabolically active irregular mass is noted averaging 3x2.5 of SUV max 7.4 as well as related lower outer FDG avid parenchyma of SUV max 2.5 . this is associated with low FDG avid thickened skin reaching 1 cm of SUV max 1.**



- **Locoregional:** This is associated with locoregional level I left axillary single FDG avid lymph node averaging 2.5 cm of SUV max 9.4 .
- **Distant:** Liver shows heterogeneous FDG uptake with segment III foci of SUV max 5 with no corresponding non contrast CT finding...for further Triphasic CT/MRI
- **Otherwise no other** No distant FDG avid metastatic deposits could be noted.

[[DOCTYPE:PET_SCAN]]

Name : Alia Abdul Salam Sayyid Nasr
Date : 29-12-2025 10:01
Branch : Nasrcity
Medical No. : 248576
Ref.Physician Prof : Goustav Roussey Egypt
Ins.Company : التأمين الصحي



Head and Neck:

- Normal diffuse brain activity with no hypermetabolic brain lesions.
- Physiologic FDG uptake is seen in the nasopharynx, oropharynx, salivary glands and larynx, showing normal appearance.
- Physiologic FDG uptake is seen in submandibular and parotid salivary glands, showing normal appearance.
- Normal appearance of the neck muscles & vessels.
- No hypermetabolic cervical lymphadenopathy detected.
- No FDG avid neck soft tissue mass lesions.

Chest:

- *Left lower lobe anterior segment Non FDG avid atelectatic bands.*
- No hypermetabolic mediastinal or axillary lymphadenopathy.
- No FDG avid lung nodules or masses detected.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart, showing normal shape.

Abdomen and Pelvis:

- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.

PET_SCAN
[[DOCTYPE:PET_SCAN]]

Name : Alia Abdul Salam Sayyid Nasr	**Date : 29-12-2025 10:01**
Branch : Nasrcity	**Medical No. : 248576**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : التامين الصحي**

- No FDG avid hepatic lesions , No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals with no FDG avid lesions detected.
- No hypermetabolic abdominal or pelvic lymphadenopathy.
- No FDG avid ascites or abdominal collections noted.
- No metabolically active pelvic lesions.

Musculoskeletal:

- ***Right femoral internal fixations is noted.***
- No osteolytic or sclerotic FDG avid lesions could be noted.
- Spondylodegertaive changes of the dorso lumber vertebrae.

Conclusion:

- Left breast FDG avid mass with related FDG avid parenchyma associated with thickened low FDG skin, single level I locoregional axillary lymph node for further Triphasic CT / Triphasic MRI study.
- No distant metastasis could be noted.

Thank you for referral,

DIAGNOSTIC CENTER
PET-Scan
[[DOCTYPE:PET_SCAN]]



جمهورية مصر العربية
بطاقة تحقيق الشخصية

عليه
عبدالسلام سيد نصر
٦ عبدالسلام المنسى-كفرطهرمس -فيصل
بولاق الدكرور - الجيزه

١٩٥٨/٠٦

٢٥٨٠٦٠٣٢١٠٠٠٦٥

LM6795055

DMS

موصى فيسجو

GUSTAVE ROUSSY
INTERNATIONAL

وستاف روسي
ترناشونال

ECHO REPORT

NAME : غالية محمد السلام

DATE: 27/9/2026

ID NUMBER : /

SEX :

AGE:

EF	62%	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	4.6	3.5-5.8cm	AO.R	2.1	3-3.7cm
LVESD	2.3	2-4cm	LA	3.5	2-4cm
FS	28%	25-45%	RV		0.7-2.3com(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions Normal

LV Wall Thickness Normal

LV Diastolic Function GI

SWMA No RWMA

LA and Ao Root Dimensions Normal

Mitral Valve Sclerotic E mild MR

Aortic Valve N

Tricuspid Valve trivial TR

Pulmonary Valve

Right Side and Systolic Function TAPSE: 2.1

Intracardiac Masses and Effusion

DR.

Echo
[[DOCTYPE:ECHO]]